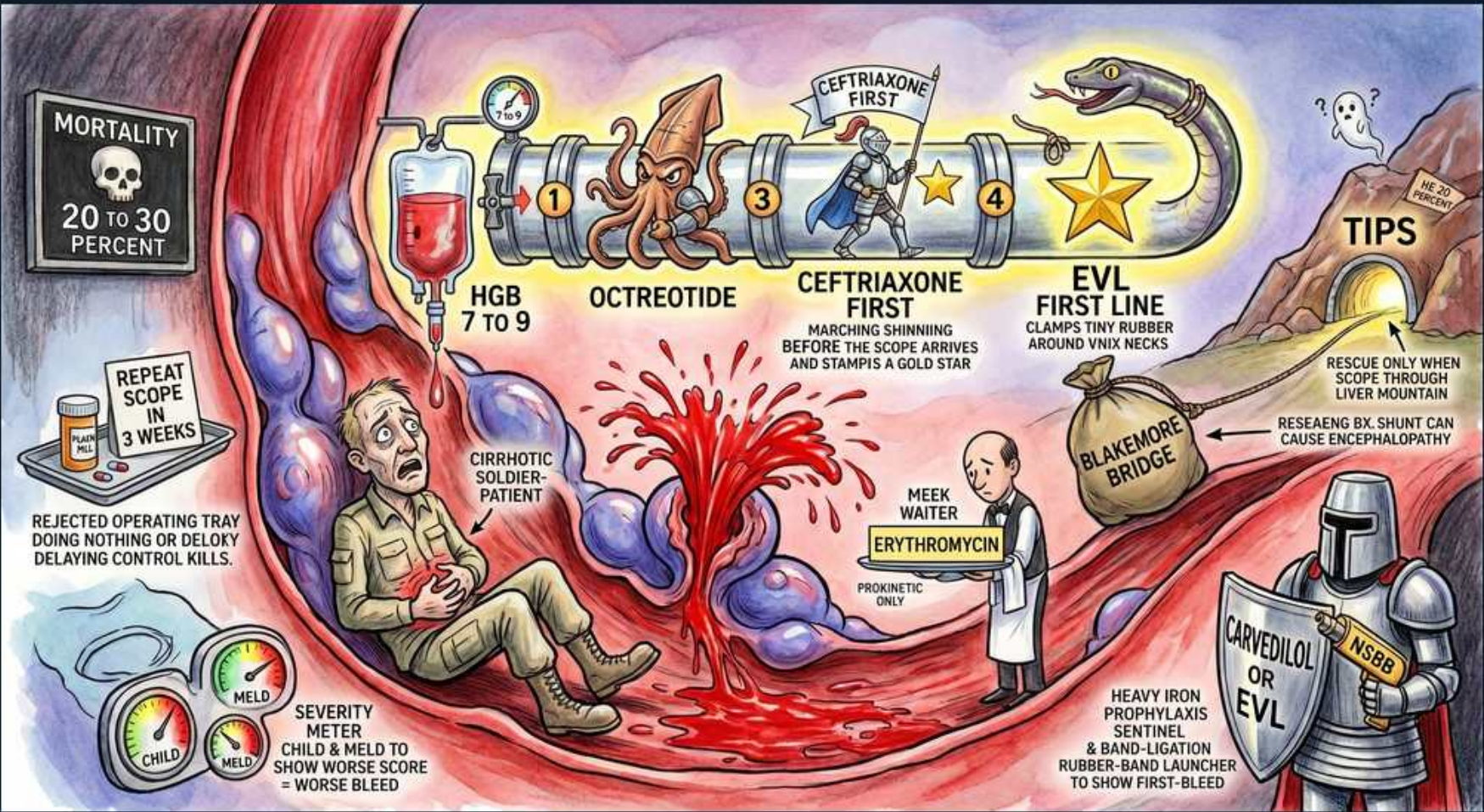


Cirrhosis — Variceal Bleeding



1. Mortality 20-30%

WHAT YOU SEE

A skull plaque reading 'MORTALITY 20 TO 30 PERCENT' on the upper-left wall.

WHAT IT ENCODES

Each acute variceal bleed carries ~20-30% mortality, so this is a true emergency requiring a parallel resuscitation bundle, not a stepwise approach. Variceal hemorrhage is the most lethal complication of portal hypertension. Risk of rebleed is highest in the first 5 days. Child-Pugh class and MELD predict who dies.

2. Hgb 7-9 (restrictive)

WHAT YOU SEE

An IV blood bag labeled 'HGB 7 TO 9' near step 1 of the pipeline.

WHAT IT ENCODES

Use a RESTRICTIVE transfusion strategy targeting Hb 7-9 g/dL; over-transfusion raises portal pressure and worsens bleeding. The classic Villanueva trial showed restrictive transfusion improved survival in acute upper GI bleed. Avoid driving Hb above 9. Also correct volume with crystalloid but avoid over-resuscitation.

3. Octreotide

WHAT YOU SEE

An octopus/squid at pipeline step 2 labeled 'OCTREOTIDE'.

WHAT IT ENCODES

Octreotide or somatostatin causes splanchnic vasoconstriction, lowering portal inflow and pressure to slow bleeding. Start as soon as variceal bleed is suspected and continue 3-5 days. Bolus then infusion. It is an adjunct to endoscopy, not a replacement.

4. Ceftriaxone FIRST (gold star)

WHAT YOU SEE

A banner 'CEFTRIAXONE FIRST' with a marching knight and a gold star at step 3.

WHAT IT ENCODES

Prophylactic ceftriaxone is given to EVERY cirrhotic with GI bleed BEFORE endoscopy; antibiotics reduce infection, SBP, rebleeding AND mortality (gold star). Typically ceftriaxone 1 g IV daily for up to 7 days. Cirrhotics with bleeding have high rates of bacterial translocation. This is one of the few mortality-reducing steps.

5. EVL first-line (gold star)

WHAT YOU SEE

A gold star at step 4 with 'EVL FIRST LINE' clamping rubber bands around snake necks.

WHAT IT ENCODES

Endoscopic variceal band ligation (EVL) is first-line hemostasis for esophageal varices (gold star), performed within 12 hours. Bands strangle the varix; superior to sclerotherapy. Repeat sessions every 2-4 weeks until eradicated for secondary prophylaxis. For gastric varices use cyanoacrylate glue injection instead.

6. Erythromycin = prokinetic only

WHAT YOU SEE

A 'MEEK WAITER' holding an 'ERYTHROMYCIN — Prokinetic Only' card.

WHAT IT ENCODES

Erythromycin 250 mg IV ~30-120 min before scope is a PROKINETIC that empties the stomach for a clear endoscopic view — it is NOT the prophylactic antibiotic. The protective antibiotic that lowers mortality is ceftriaxone. Don't confuse the two; erythromycin has no mortality benefit here.

BOARD TRAP

"Erythromycin is the prophylactic antibiotic for variceal bleed" → wrong; erythromycin is only a prokinetic to clear the gastric view. The mortality-reducing prophylactic antibiotic is ceftriaxone.

7. TIPS = rescue only

WHAT YOU SEE

A tunnel through a mountain labeled 'TIPS' on the right with a 'rescue only' note.

WHAT IT ENCODES

TIPS (transjugular intrahepatic portosystemic shunt) is RESCUE therapy only when endoscopy fails to control bleeding (or early pre-emptive TIPS in selected high-risk Child B/C). It diverts portal blood to systemic circulation, dropping portal pressure, but can precipitate hepatic encephalopathy and worsen liver function. Not the first move.

BOARD TRAP

"Go to TIPS first" → wrong; TIPS is only after endoscopic failure (or selected high-risk early-TIPS), because the shunt can cause encephalopathy and is invasive.

8. Blakemore = bridge only

WHAT YOU SEE

A sandbag labeled 'BLAKEMORE BRIDGE' between the body and TIPS tunnel.

WHAT IT ENCODES

Sengstaken-Blakemore / balloon tamponade is a TEMPORARY bridge (max ~24h) to control massive bleeding until definitive endoscopy or TIPS. Risks esophageal necrosis/rupture and aspiration; requires airway protection. Never definitive therapy.

9. Repeat scope in 3 weeks (X)

WHAT YOU SEE

A 'REPEAT SCOPE IN 3 WEEKS' pill bottle on a rejected tray, lower left.

WHAT IT ENCODES

Delaying intervention — 'observe', 'repeat scope in 3 weeks', or doing nothing — kills, because rebleed mortality is high in the first days. Acute bleeding demands the immediate bundle now. Scheduled surveillance scope is for prophylaxis, not active hemorrhage.

BOARD TRAP

"Repeat scope in 3 weeks / observe / delay" → wrong; delay kills in active variceal bleeding. Resuscitate, give octreotide + ceftriaxone, and band-ligate now.

10. Severity meter (Child/MELD)

WHAT YOU SEE

Two gauges labeled 'CHILD' and 'MELD' on a 'SEVERITY METER', lower left.

WHAT IT ENCODES

Child-Pugh (bilirubin, albumin, INR, ascites, encephalopathy) and MELD (bilirubin, INR, creatinine) score liver severity and predict mortality and rebleed risk. Higher score = worse bleed and worse outcome; helps select early-TIPS candidates and transplant priority.

11. NSBB / EVL prophylaxis

WHAT YOU SEE

An armored 'NSBB' knight beside a banner 'CARVEDILOL OR EVL', lower right.

WHAT IT ENCODES

Primary and secondary prophylaxis of variceal bleeding = non-selective beta-blocker (carvedilol or propranolol/nadolol) OR endoscopic band ligation; secondary prophylaxis often combines NSBB + EVL. NSBBs lower portal pressure (target HR ~55-60 or 25% drop). Carvedilol is now preferred for its added alpha-1 effect.